



OFFICIAL

Assessment Requirements for Mental Health Consumers Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Kalamunda Group	Mental Health	Medical, Nursing, Allied Health, Clerical

2. Purpose

A comprehensive mental health assessment allows clinicians to gather information in a culturally responsive manner that incorporates their reason for presentation, bio psychosocial history and level of priority based on risk. The assessing clinician will formulate a suitable treatment, support and discharge plan in collaboration with the consumer, their carer and other relevant stakeholders involved in the consumer's care. These assessments assist clinicians to determine ongoing involvement with the service and/or referral to other services.

3. Procedure

Initial Inpatient, CRHTT and Community Assessment procedure:

Dependent on the location at which the consumer presents, different assessment forms are required to ensure the most appropriate information is captured on that presentation. Information to be accessed as part of the face to face or telephone assessment includes the medical record, PSOLIS contacts, crisis and management plan, WebPAS record, information from other involved stakeholders such as Next of Kin; Carer; Private Psychiatrist; private Psychologist and General Practitioner. All assessments will adhere to the sexual safety standards that define appropriate behaviour for the service setting. If a consumer is assessed as being vulnerable to breaches of sexual safety, this assessment must be documented within the admission clinical documentation, progress notes and treatments support and discharge plans.

4. Referrals to inpatient units

All referrals to Mental Health inpatient units are to have a minimum level of information including:

- History of presenting complaint
- Past psychiatric and medical history
- Current list of medications
- Mental state examination including formulation and plan
- Any physical problems that need follow up and monitoring within the first 24 hours of admission to a ward

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- Any risks identified at the time (self, others, dependents or property etc.). A copy of the *MHA 2014* forms that are in place.
- Consumers admitted via ED must have Medical Clearance form completed, and it must indicate that the consumer is cleared for transfer to the Inpatient Unit prior to transfer occurring.

The information is to be documented as per WA Health State-wide Standardised Clinical Documentation for (SSCD) for Mental Health Services OD 0526/14. [State-wide Standardised Clinical Documentation for Mental Health Services](#)

For referrals within EMHS, the above requirements for inpatient admission are to be detailed in the highlighted sections of the Mental Health Assessment Form (MHAF). Referrals from other HSPs may be completed on either the MHAF or Triage Form as per [EMHS Mental health patient Flow and Bed capacity Framework](#).

When health records are volumised, the most current MHAF, RAMP, TSDP and relevant paperwork is scanned into DMR.

5. Inpatient assessment and/or review

Step 1- On admission the admitting clinician completes the following assessments with the presenting information, consumer and carer as appropriate.

- Adult Observation and Response Chart (AKMR140.3)
- Alcohol, Smoking, Substance Involvement Screening Tool - ASSIST (AKMR27)
- Risk Assessment and Management – RAMP, including risk of being vulnerable to having their sexual safety breached or compromising someone else's safety
- Alerts (PSOLIS)
- Patient Safety Plan (PSP)
- Falls Risk Assessment Management Plan - FRAMP (AKMR24)
- Mental Health Assessment Form (SMHMR902)
- National Outcome and Case-mix Collection - NOCC
- Physical Examination (SMHMR903)
- Physical Appearance (SMHMR904)
- Urine Drug Screen – UDS
- Treatment Support and Discharge Plan – TSDP (PSOLIS)
- Skin and Pressure Injury Assessment (AKMR121.9)
- Patient Property Form as per AKG Patient Property in Mental Health Wards Procedure
- Screening for Family and Domestic Violence (FDV 950) NB: When domestic violence is indicated this will be referred to the team Social Worker who will then complete the Assessment Family and Domestic Violence Screen (FDV951) in conjunction with the team doctor
- Admission Checklist (PAKMR48.7)

Step 2 - The following observations are to be undertaken for the first 48 hours from admission; and to be continued when clinically indicated and as per [Observations: Safe and Supportive](#):

- Adult Observation and Response Chart (AKMR140.3)
- Safe and Supportive Observation Chart (AKMR147)
- Bowel Chart (AKMR 141)

Step 3 – Any outstanding/incomplete assessments must be documented in iSOFT Clinical Manager and raised at each shift handover and at the daily stand-up meeting.

CRHTT assessment – Crisis Resolution Home Treatment Team

CRHTT suitability assessment (pre-admission)

CRHTT complete the following on suitability assessment prior to admission:

- RAMP on web-PSOLIS
- Mental Health Assessment Form completed on web-PSOLIS

Step 1- On admission the admitting clinician completes the following assessments with the presenting information, consumer and carer as appropriate.

- CRHTT consumer admission form PAKMR6.3
- CRHTT consent to release/obtain information AKMR30.2
- Metabolic Screening Assessment Chart (EMR313210)
- Alcohol, Smoking, Substance Involvement Screening Tool - ASSIST (AKMR27)
- Risk Assessment and Management Plan – RAMP (SMHMR905) web-PSOLIS
- Enter Alerts as identified (PSOLIS)
- GP Letter sent
- CRHTT welcome pack
- Mental Health Assessment Form (SMHMR902)
- National Outcome and Case-mix Collection - NOCC
- Physical Examination (SMHMR903)
- Physical Appearance (SMHMR904)
- Treatment Support and Discharge Plan – TSDP (SMHR902) web-PSOLIS
- Assessment Family and Domestic Violence Screen (FDV951) completed on admission
- CRHTT Admission Checklist
- Bowel Chart (AKMR 141) (as identified for CRHTT)
- Falls Risk Assessment Management Plan - FRAMP (AKMR24) (as identified for CRHTT)
- Skin and Pressure Injury Assessment (AKMR121.9) (as identified for CRHTT)
- Urine Drug Screen – UDS (as identified for CRHTT)

Step 2 Any outstanding/incomplete assessments must be documented in iSOFT Clinical Manager and raised at each shift handover.

6. Inpatient medical reviews

Consumers on Yorgum will be reviewed by their treating team doctor(s) at least once daily. Sedative medications must be prescribed judiciously and monitored according to observed protocols. Consumers on Moodjar, Karri and Banksia wards will have documented reviews by their treating team doctor(s) at a minimum of every two days.

7. CRHTT Consultant Review

Consultant review on admission to CRHTT with further medical reviews as identified by CRHTT Registrar within the 14-day admission.

8. Community assessments for all teams

Assessments for consumers in the community are to be completed within four weeks of activation:

- Alcohol, Smoking, Substance Involvement Screening Tool - ASSIST (AKMR27)
- Risk Assessment Management Plan (web-PSOLIS)
- Patient Safety Plan (PSP)
- Treatment Support Discharge Plan– TSDP (web-PSOLIS)
- Liverpool University Neuroleptic Side Effect Rating Scale – LUNSERS (PAKMR179)
- Mental Health Assessment Form – MHAF (SMHMR902)
- National Outcome and Case-mix Collection - NOCC
- Metabolic Screening Assessment (AKMR 1695)
- Screening for Family and Domestic Violence (FDV 950) NB: When domestic violence is identified, the Assessment Family and Domestic Violence Screen (FDV951) is to be completed by the Care Coordinator in conjunction with the team doctor.

9. Ongoing assessment – mental health assessment form and clinical review in community

- The Mental Health Assessment Form - MHAF must be reviewed at least every twelve months undertaking a comprehensive assessment involving the use of the MHAF (SMHMR902) and Risk Assessment and Management Plan- RAMP to identify consumer risk factors, including risk of being vulnerable to having their sexual safety breached or compromising someone else's safety
- The clinical review will occur every three months in community document the consumer's diagnosis, treatment plan, and physical health and follow up arrangements.
- Prior to admission to inpatient service or into our ED services, even with a MHAF completed within last 12 months, the section in MHAF on demographics inclusive of current General Practitioner (GP), current presentation and formulation inclusive of mental state assessment, current treatment and plan must be updated to inform the receiving services of the changes in presentation.

Step 4 - If the consumer is to be admitted then the MHAF is to be completed as soon as practicable.

10. Communication with the identified General Practitioner

Communication with the consumer's identified GP is of the utmost importance to ensure the consumer is receiving comprehensive care when specialist and primary health sector are involved in the care of the consumer.

Essential points at which the identified GP must receive written communication from the Armadale Mental Health Service are:

10.1 Inpatient

- At admission advising of the admission.
- At discharge via the NACS discharge summary.

10.2 Community

Medical staff have a role in communication with the GP at the following points:

- Initial assessment, outlining reason for referral, impression, treatment, support and discharge plan and follow up arrangement. Prescribed medication changes, as documented on Best Practice Prescription program
- Significant changes in mental state examination (MSE)
- Changes in physical health
- Provision of Mental Health Tribunal reports, with the permission of the consumer
- Care Coordinator only to provide written correspondence to GP if consumer has not been reviewed by a doctor during their episode of care or has not engaged with the community service.
- The patients GP should be updated routinely every 3 months even if none of the above has occurred

10.3 CRHTT

On admission to the service CRHTT clinicians will send out the GP letter to inform of admission to service with consent from consumer. The consumer is also encouraged to make a GP appointment during the period of admission with CRHTT, CRHTT clinicians will endeavour to support the consumer with making / escorting to the appointment during the 14-day admission period.

11. Legislative context

The following documents are required to give effect to this procedure:

- [Mental Health Act 2014](#)
- HDWA
 - [Statewide Standardised Clinical Documentation for Mental Health Services Procedure](#)
 - [Statewide Mental Health Bed Access, Capacity and Escalation Policy](#)
- EMHS
 - [Mental Health Bed Access, Capacity, and Escalation Framework](#)
 - [Discharge Communication \(Written\)](#)

12. Supporting information

The following documents support the implementation of this procedure:

- HDWA
 - [Triage to Discharge](#)
 - [Aboriginal Health and Wellbeing Policy](#)
- EMHS
 - [Mental Health Consumers who Decline Assessment](#)
- AKG
 - [Discharge from the Mental Health Inpatient wards and Community Mental Health Teams](#)
 - [AHS Observations-Physiological Procedure](#)

- [LUNTERS \(Liverpool University Neuroleptic Side Effect Rating Scale\) – Using for Monitoring Neuroleptic Side Effect Guideline](#)
- [Consumer Safety Planning Procedure](#)
- [Identification of Patients in the Armadale Mental Health Service Procedure](#)
- [Mental Health Consultation and Liaison Procedure](#)
- [National Outcome and Case mix Collection \(NOCC\) Procedure](#)
- [Observations: Safe and Supportive Procedure](#)
- [Communicating for Safety](#)
- [Patient Property in Mental Health Wards Procedure](#)

13. Compliance, monitoring and evaluation

Compliance against this policy will be monitored by the Mental Health Safety and Quality Committee through auditing as per AKG Mental Health Service Audit Schedule and monitored to ensure action plans arising from these audits are completed in the appropriate time frame.

14. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 – Action 5.3 Clinicians use organisational processes from the partnering with Consumers Standard when providing comprehensive care to:
- Standard 5 - Action 5.7 The health service organisation has processes relevant to the patients using the service and the services provided:
- Standard 5 - Action 5.10 Clinicians use relevant screening processes:
- Standard 5 - Action 5.12 Clinicians document the findings of the screening and clinical assessment processes, including and relevant alerts, in the healthcare record.
- Standard 6 - Action 6.10 The health service organisation ensures that there are communication processes for patients, carers and families to directly communicate critical information and risks about care to clinicians.
- Standard 6 - Action 6.11 The health service organisation has processes to contemporaneously document information in the health care record, including:

National Standards for Mental Health Services (2010):

- Standard 2 - Criteria 2.11 The MHS conducts risk assessment of consumers throughout all stages of the care continuum, including consumers who are being formally discharged from the service, exiting the service temporarily and / or transferred to another service.

[Chief Psychiatrist's Standards for Clinical Care](#)

15. Glossary

The following definitions are relevant to this policy.

Term	Definition
Consumers	Any person who identifies as having a current or past lived experience of psychological or emotional issues, distress or problems, irrespective of whether they have a diagnosed mental illness and/or have received treatment.
CRHTT	Crisis Resolution Home Treatment Team

16. Document owner

Service Director Mental Health

Enquiries relating to this procedure may be directed to:

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This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V7	01 12 2025	01 12 2028	Full review; significant changes throughout documents and updated links.

17. Authorisation

This procedure has been authorised and issued by:

Authorised by	Karen Elliott – Service Director Mental Health
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